BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

Check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Blood / kidney / liver	full blood count (FBC), renal and liver profile	Confirm treatment-day results	FBC, renal and liver tests before each cycle; confirm Day 8 review	Review unresolved abnormalities	Individual clinical plan
Heart / breathing	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/breathing symptoms	Clinical indication
Cancer review	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks	dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) before 5-FU	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00378 v6 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

CMF · intravenous (IV) · 28-day schedule: treatment and checks

Adjuvant when anthracyclines are unsuitable, or metastatic breast cancer

All three medicines are given intravenously on Days 1 and 8 of each 28-day cycle. Six cycles for adjuvant treatment; metastatic treatment continues while beneficial and tolerated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00378	Confirm exact regimen	CMF · IV · 28-day schedule	All three medicines are given intravenously on Days 1 and 8 of each 28-day cycle. Six cycles for adjuvant treatment; metastatic treatment continues while beneficial and tolerated.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00378	Confirm adjuvant or metastatic setting	Follow the multidisciplinary team (MDT) sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00378	FBC, renal and liver profile	Confirm treatment-day results	FBC, renal and liver tests before each cycle; confirm Day 8 review	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00378	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/breathing symptoms	Clinical indication
Cancer review NCCP 00378 · EARLY · METASTATIC · MARKERS	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks NCCP 00378 · BONE	DPD before 5-FU	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1 Baseline review

Before treatment

Adjuvant when anthracyclines are unsuitable, or metastatic breast cancer. Confirm DPD testing before the first fluorouracil dose. Kidney impairment can affect interpretation of uracil-based testing; ask the oncology team to review the result and testing method.

Ask / **act:** Confirm source eligibility and results.

Sources: NCCP 00378

2 Treatment-day check

Before each planned dose

NCCP specifies FBC, kidney and liver tests before each cycle. The unit must also confirm the Day 8 assessment and oral-dose instructions.

Ask / **act:** Confirm active medicines and whether a dose is held.

Sources: NCCP 00378

3 Support plan

At every phase or day change

All three medicines are given intravenously on Days 1 and 8 of each 28-day cycle. Six cycles for adjuvant treatment; metastatic treatment continues while beneficial and tolerated.

Ask / **act:** Choose the matching support script; do not merge phases.

Sources: NCCP 00378

4 Response and follow-up

Agreed review points

The treatment setting changes the scan plan. After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to look for distant recurrence in a person without symptoms or clinical concerns. For metastatic disease, record the disease sites and agree the scan type and review interval before treatment. Review response, symptoms and toxicity together; worsening symptoms may need earlier assessment.

Ask / **act:** Review symptoms, results and patient priorities together.

Sources: NCCP 00378 · EARLY · METASTATIC · MARKERS

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.18